

Workplace rehabilitation standard

Human Resources Standard (QH-IMP-401-5)

Statement

This standard identifies the minimum requirements that evidence the implementation of the Health, Safety and Wellbeing policy for managing injuries and illnesses related to active workers compensation cases and all other workplace rehabilitation cases. This standard supports recovery in the workplace and Queensland Health's commitment to ensuring and improving the physical and psychological health, safety and wellbeing of workers.

Summary of this standard

- Commitment to improving wellbeing and rehabilitation outcomes for injured workers.
- Local processes detailed in rehabilitation procedure/s, including the 4 stages of rehabilitation, identification of accountability area roles to fulfill key responsibilities for workplace rehabilitation, a workers rights in relation to workplace rehabilitation and the appropriate dispute resolution process to follow.
- Local workplace rehabilitation case management processes which meet legislative requirements, inclusive of workers signed authorisation prior to exchanging confidential information with third parties, a file for each worker undertaking workplace rehabilitation containing copies of all documentation relevant to the injury and rehabilitation case, and records storage and retention as stipulated by legislation.
- Availability of sufficient and appropriately qualified rehabilitation and return to work coordinators (RRTWCs) to support injured workers through workplace rehabilitation, with required RRTWC details submitted on an annual basis to WorkCover Queensland.
- A mandatory, current workers' compensation insurance policy that covers all accountability area workers, is held with WorkCover Queensland.
- Recognition of the health benefits of good work through stay at work and return to work practices that support meaningful workplace rehabilitation plans, with rehabilitation goals prioritised according to the return to work (RTW) hierarchy.
- Promotion of line manager responsibilities in workplace rehabilitation, including identification of suitable duties and early and ongoing communication with workers.

Application

This standard applies to all accountability areas and includes employees within the Department of Health (the department) and Hospital and Health Services (HHSs), in relation to the provision of workplace rehabilitation for active workers compensation and all other cases, including closed workers compensation claims, income protection and health management. This document does not apply to the Queensland Ambulance Service and instead, reference should be made to the relevant Queensland Ambulance Service

policy/procedure/guideline. Accountability areas will also develop local procedures, outlining how workplace rehabilitation is implemented, to operationalise this standard.

Requirements

1 Principles

Queensland Health is committed to providing workplace rehabilitation for injured/ill workers, irrespective of the workers entitlement to compensation. Workplace rehabilitation is the system for managing active workers compensation cases and all other cases to support staff with injury and illness at work.

The primary goal of workplace rehabilitation shall be to support the injured/ill worker to return to their substantive role within their usual work area, where reasonably practicable.

Queensland Health shall maintain an effective workplace rehabilitation system, in accordance with the *Workers' Compensation and Rehabilitation Act 2003* (the act), which meets the Guidelines for Standard for Rehabilitation, and/or other scheme directions as set out in section 329A of the act.

Queensland Health is committed to preventing workplace psychosocial hazards as per the requirements of *Managing the Risks of Psychosocial Hazards at Work Code of Practice 2022*. It is recognised that psychosocial hazards may be associated with workplace rehabilitation, and these can be mitigated by adopting a best practice workplace rehabilitation system.

The workplace rehabilitation system recognises and addresses potential psychosocial hazards that may arise from changes experienced at certain stages of workplace rehabilitation. These may include, but are not limited to, altered work hours or duties, altered work environment and/or working relationships while undertaking suitable duties, communication breakdown between key stakeholders, disagreement on suitable duties tasks or a perceived lack of empathy, autonomy, fairness or control.

Queensland Health's workplace rehabilitation system shall align with the principles outlined in the Royal Australasian College of Physicians' evidence-informed paper: *It Pays to Care - Bringing evidence-informed practice to work injury schemes helps workers and their workplaces* (April 2022). This national policy position promotes fair, constructive, and supportive recovery journeys for injured and ill workers and is operationalised through local rehabilitation processes and procedures.

Workplace rehabilitation processes shall embed a person-centred and trauma informed approach to provide support for injured and ill workers in Queensland Health.

Queensland Health is committed to supporting a reframed relationship with Aboriginal peoples and Torres Strait Islander peoples in accordance with Chapter 1 Part 3 of the *Public Sector Act 2022*. Workplace rehabilitation will acknowledge unique experiences and cultural needs of injured and ill workers for improved rehabilitation outcomes.

The workplace rehabilitation system shall encompass the rights and protections set out in the *Anti-Discrimination Act 1991* and the *Human Rights Act 2019*, to ensure that the principles of freedom, respect, equality and dignity underpin workplace rehabilitation interactions and actions and decisions occur in a way that is compatible with human rights.

A decision maker has an obligation under the *Human Rights Act 2019* to act and make decisions in a way that is compatible with human rights and give proper consideration to human rights.

Queensland Health's workplace rehabilitation system shall align with the vision and guiding principles of the *National Return to Work Strategy 2020-2030* and acknowledges the influence that employers and workplaces have on rehabilitation outcomes.

The rehabilitation approach shall support the health and wellbeing benefits of good work, in accordance with the principles outlined in the Royal Australasian College of Physicians' position statement: *Realising the health benefits of work*.

2 Roles and Responsibilities

2.1 The employer / executive leader of the accountability area shall:

- Ensure the implementation of the health, safety and wellbeing policy in conjunction with this standard and supporting documents within their accountability area.
- Ensure workers are insured, and remain insured, with WorkCover Queensland for all work-related injuries and illnesses and not take action to avoid the workers compensation process as per section 46A of the act.
- Ensure workers are aware of compensable entitlements for work-related injury/illness.
- Ensure sufficient and appropriately qualified rehabilitation and RRTWC are available to support injured/ill workers through provision of workplace rehabilitation.
- Enable the implementation of suitable duties for all active workers compensation cases and reasonable adjustment solutions, where reasonably practicable, with appropriate allocation of resources and support, in accordance with section 228 of the act and the *Reasonable Adjustment HR Policy G3*.

2.2 The line manager, or delegate, shall:

- Support the rehabilitation of injured/ill workers by ensuring the workplace rehabilitation standard and local workplace rehabilitation procedures and processes are followed.
- Ensure access to immediate first aid in the event of injury/illness occurring at work.
- Ensure all workplace incidents are recorded and investigated in accordance with the requirements of local incident reporting procedures and Work Health and Safety Incident response standard.
- Ensure that injured/ill workers are made aware of the workplace rehabilitation support available, including availability of suitable duties and the required medical information to consider reasonable adjustment, where applicable.
- Contact the injured/ill worker to offer workplace rehabilitation support and explain the involvement of the RRTWC as soon as practicable after reporting of the injury/illness.
- Notify the workers designated RRTWC as soon as practicable, to facilitate a referral, as required, to support workers with workplace rehabilitation and stay at work or return to work processes.
- Maintain regular, transparent and timely communication, in the manner as agreed, with the injured/ill worker throughout the rehabilitation process, whether at work or off work,

regarding the workers recovery and rehabilitation progress as well as communication of any workplace changes and available supports.

- The immediate line manager is best placed to be the primary contact for the worker wherever possible and facilitate the injured workers on-going connection with the workplace.
- Where alternate work duties are required, identify and actively seek, with support from the RRTWC if needed, meaningful suitable duties, and ensure suitable duties are made available wherever reasonably practicable.
- Ensure suitable duties considerations are documented for all accepted workers compensation cases. In rare instances, if it is determined that it is not practicable to provide suitable duties to the worker, the evidence relied on to support that opinion must be provided to the insurer, as per s228(2) of the act.
- Monitor implementation of the suitable duties program, to ensure duties provided comply with the injured/ill workers current medical capacity, ensuring the health and safety of the worker and others in the workplace.
- Provide the Payroll Injury and Rehabilitation Claims Team (PIARCT) with accurate information pertaining to the injured/ill workers hours of work during the rehabilitation process, via the approved form/s and within required payroll cut off schedule. Further detail can be found at <https://qheps.health.qld.gov.au/payroll-assistance/payroll-services/piarct>.
- Inform the worker of the appropriate dispute resolution process to follow in such cases where the worker does not agree with elements of a proposed workplace rehabilitation plan or suitable duties program.
- Ensure the rights of workers-on-workers compensation claims, are upheld, including support to choose their treating medical practitioner and who is present during medical treatment for the injury, as per s208B of the act.

2.3 The Rehabilitation and Return to Work Coordinator (RRTWC) shall:

- Support the rehabilitation of injured/ill workers by ensuring this Workplace rehabilitation standard and local workplace rehabilitation procedures are followed.
- Ensure that an injured/ill worker who requires workplace rehabilitation for either a work or non-work-related injury/illness, is contacted for workplace rehabilitation support, as soon as practicable after notification of the injury/illness or request for support.
- Request the injured/ill workers signed authorisation to communicate with, and obtain information from the workers treating doctor and/or other health practitioners, for the purposes of planning and implementing workplace rehabilitation.
- Outline obligations of all parties under the act, where a workers compensation claim has been made.
- Provide coordination of an injured/ill workers workplace rehabilitation and maintain effective communication with key stakeholders in the workplace rehabilitation process.
- Review and provide input into rehabilitation and return to work plans for accepted workers compensation claims, these must be provided by the insurer within 10 business days of claim acceptance.

- Communicate with and inform the treating practitioner of the workers substantive position and the availability of suitable duties, to enable consideration of tasks/duties that will meet the injured/ill workers functional capacity to safely remain at, or return to the workplace.
- Advise and support the line manager to identify meaningful suitable duties that comply with the injured/ill workers current medical capacity and support the line manager to implement and monitor the suitable duties program, as required.
- Document all considerations, actions and decisions regarding suitable duties. For a worker with an accepted workers' compensation claim, written documentation, including the steps taken or the inquiries made to reach a decision that it is not practicable to provide suitable duties, must be provided to the insurer, in accordance with s228(2) of the act.
- Inform the worker of the appropriate dispute resolution process to follow in such cases where the worker does not agree with elements of a proposed workplace rehabilitation plan or suitable duties program.
- Ensure factual and timely case file notes are recorded for each workplace rehabilitation case.
- Ensure legislative compliance is maintained with respect to privacy, confidentiality and use of worker information and workers compensation information.
- Ensure legislative compliance is maintained with respect to rehabilitation file records, storage and retention.
- Maintain and develop appropriate injury management skills and knowledge.

2.4 The injured/ill worker shall:

- Report a workplace incident in accordance with the requirements of local incident reporting procedures and the Work health and safety Incident response standard.
- Notify the line manager of the emergence of injury/illness that may impact work capacity, so that workplace rehabilitation support can be considered. This includes, injury/illness and pre-existing medical conditions not covered by workers' compensation.
- Notify the line manager of lodgement of a workers' compensation claim application or an income protection benefits claim application and submit a copy of the insurer-approved medical certificate/capacity certificate.
- Ensure updated and current medical certificates are provided to the line manager and RRTWC in a timely manner.
- Ensure appropriate leave application/s are submitted for accurate payments and to prevent overpayments, which will result in an overpayment recovery process.
- Ensure medical certification remains current during participation in workplace rehabilitation and any absence from the workplace, providing copies to the line manager or agreed workplace contact.
 - Should a medical certificate outlining commencement of work capacity not be provided sufficiently in advance, the work unit, in consultation with the RRTWC or other appropriate stakeholders, will need to determine if suitable duties can be provided within the nominated timeframe.

- Following provision of medical information, further liaison with the treating doctor may be required before a RTW program can commence.
- During discussion and consideration of available suitable duties and/or agreement of reasonable adjustment, appropriate leave entitlements should be accessed.
- Make every effort to stay at work/return to work in accordance with medical capacity.
- Contribute constructively to RTW planning.
- Ensure duties performed on a suitable duties program comply with medical restrictions.
- Actively communicate with the RRTWC and line manager/other nominated workplace contact at agreed intervals via the agreed method.
- Engage in the overall workplace rehabilitation process, noting workers with active workers compensation claims may impact their entitlement to compensation if they fail to participate in rehabilitation, as per s232 of the act.

2.5 The Payroll Injury and Rehab Claims Team (PIARCT) shall:

- Process all accepted workers compensation claim payment data and leave processing for graduated return to work. Further detail can be found at <https://qheps.health.qld.gov.au/payroll-assistance/payroll-services/piarct>.

3 Rights of key parties

A workers right to be informed of the workers' compensation scheme available for work-related injury and illness is recognised. Workers should be informed about their right to make an application for compensation, choose their treating medical practitioner, who is present during treatment, their workplace rehabilitation provider, consultation requirements in development of rehabilitation and return to work plans, and the right to seek additional advice and/or legal support, as needed.

A workers right to disagree with elements of a proposed workplace rehabilitation plan is recognised, and the line manager/RRTWC shall inform the worker of the appropriate dispute resolution process to follow.

- The commitment to continuing with workplace rehabilitation during and after the dispute resolution process is recognised by both the worker and employer. This process is outlined in the *Individual employee grievances HR Policy E12*.

The rights of all parties to request a review or appeal of a workers' compensation or income protection insurance claim decision is recognised and supported with a commitment to continuing a workers rehabilitation program during and after any review or appeal process. Refer to the respective insurer processes for further information.

4 Workplace rehabilitation system

4.1 Key principles

The four stages for managing the rehabilitation of workers with injury/illness in the workplace are:

- Stage 1 – immediate support and administrative processes
- Stage 2 – planning for stay at work or return to work

- Stage 3 – implementing and supporting the plan
- Stage 4 – evaluating and reviewing rehabilitation outcomes.

The process flow for the workplace rehabilitation system, is outlined in attachment A.

The primary goal of workplace rehabilitation is to support the injured/ill worker to stay at work throughout their recovery or return to performing their substantive role in their usual work area, where reasonably practicable and no foreseeable risk to the worker or others exists.

Suitable duties and/or temporary reasonable adjustment to the workplace (such as modifications to work practices or duties) will be implemented for all active workers compensation claims during the rehabilitation process, where it is indicated on a medical certificate, reasonably practicable, and does not introduce unacceptable workplace risk. All other cases will be supported on a case-by-case basis. Attachment B, suitable duties workflow and the workplace rehabilitation guideline provide further detail.

Rehabilitation goals shall be prioritised according to the RTW hierarchy (see workplace rehabilitation guideline) wherever feasible, whereby the RTW goal is identified with a preference to returning the worker to their usual job in their usual work area. Where this is not possible, processes as per the *Reasonable adjustment HR policy G3* may be required.

4.2 Stage 1 – Immediate support and administrative processes

4.2.1 First response

All injured/ill workers shall be offered immediate first aid in the event of injury/illness occurring in the workplace. Access to first aid in the event of injury/illness, shall be considered for workers working away from their usual workplace at the direction of the employer.

More serious injury or illness may require activation of local emergency response procedures, and support to access immediate medical care, as needed.

Workers requiring medical treatment for a work-related injury, which is not a medical emergency, bodily fluid exposure or electric shock exposure, may access medical treatment from a general practitioner, rather than an Emergency Department, where appropriate.

Immediate workplace supports may include:

- assistance to seek suitable medical care
- provision of information about availability of suitable duties
- safe transportation from work to medical care or place of residence, as applicable
- the injured workers line manager notifying next-of-kin, if applicable.

The workplace incident shall be recorded in Queensland Health's incident management system and in accordance with the requirements of local incident reporting procedures and the work health and safety incident response standard. The workers line manager is authorised to complete incident reporting on the workers behalf when a worker is unable to do so.

When the line manager is made aware of injury/illness impacting a workers ability to perform the genuine requirements of their role the worker should be offered rehabilitation support and a request for supporting medical information may be required.

4.2.2 Workplace rehabilitation

Where medical certification supports stay at work or return to work, workplace rehabilitation shall occur as per the workplace rehabilitation process outlined in attachment A. Further guidance is available in the Workplace Rehabilitation Guideline.

A rehabilitation case file will be documented in the local case management system, for each workers rehabilitation case.

4.2.3 Early Communication

Early and regular, ongoing communication with the injured worker from both the manager (or nominated workplace contact) and RRTWC is vital to a successful rehabilitation outcome.

- Ongoing communication at agreed intervals, and method, between the injured/ill worker and key stakeholders is a key principle underlying successful rehabilitation.
- Workers who do not receive ongoing meaningful contact may develop related secondary injuries that can extend their workplace absence.

Early and ongoing communication with the treating doctor, following the workers signed authorisation, ensures the doctor is aware of workplace support for rehabilitation and any available suitable duties.

For accepted workers compensation claims, early and regular communication with the insurer can inform the development of a rehabilitation and RTW plan.

4.2.4 Worker signed authority

Queensland Health holds a duty of care to workers and others in the workplace under the *Work Health and Safety Act 2011*. Seeking medical information to assist a person to remain safe in the workplace contributes to fulfilling this obligation.

The workers signed authority to obtain/release information relating to the medical condition that is the subject of workplace rehabilitation, shall be obtained at the outset and remain in place until the workers rehabilitation case has finalised or the worker advises their employer, in writing, of their decision to revoke the authority.

Where authority is not provided by the worker, the RRTWC shall:

- Request relevant information for the purposes of planning and implementing workplace rehabilitation from the workers' compensation insurer for an accepted workers compensation claim; and/or
- Communicate with treating practitioner/s via the injured/ill worker who obtains the required medical information, in writing, from their treating practitioner/s.

4.2.5 Medical evidence

Medical evidence is relied upon to provide safe and effective workplace rehabilitation support and ensure the accountability area's duty of care for injured/ill workers and others in the workplace is maintained.

A request for supporting information/medical certification is required when workplace rehabilitation is requested by a worker or their line manager.

Where a risk to the workers (or others') health, safety or wellbeing is identified and/or when there appears to have been a change to the workers capability to undertake the genuine

occupational requirements of their role, a request for supporting information regarding work capacity may be made.

Where additional information is required in a stay at work/return-to-work process, the work area, in consultation with the injured worker and the insurer, (as relevant), may seek to engage an occupational rehabilitation provider (ORP). The ORP can conduct a worksite visit, functional capacity assessment or occupational analysis and report, providing necessary evidence and guidance on work capacity for a safe and sustainable stay at work/return to work.

4.3 Stage 2 – Planning for stay at work/return to work

The purpose of planning workplace rehabilitation is to ensure a coordinated, safe and sustainable process.

The principles and purpose of workplace rehabilitation apply to workers who stay at work without a period of total incapacitation following injury/illness, but still require structured and goal-oriented rehabilitation to regain functional capacity to return to pre-injury duties.

Workers with an accepted workers' compensation claim, must be provided with rehabilitation during the prescribed period as per s228 of the act, including the accountability area taking all reasonable steps to assist or provide the worker with suitable duties where reasonably practicable and supported by medical advice, as detailed in attachment B.

4.3.1 Suitable duties program

If a worker cannot immediately undertake their pre-injury work duties, consideration shall be given to the development of a temporary suitable duties program, as per the suitable duties process flowchart at attachment B.

It is an expectation that meaningful suitable duties will be identified and made available through the workers usual work unit, where reasonably practicable. If this is not practicable, the line manager, in consultation with the RRTWC, may liaise with other managers in different work units, to identify options, including consideration of host placement options.

If workers with accepted workers compensation claims cannot be offered suitable duties consistent with medical restrictions, all actions and considerations of suitable duties must be documented. This includes the steps taken or inquiries made to reach the determination, this evidence is relied upon to support the opinion that it is not practicable to provide suitable duties to the worker.

4.3.2 Temporary reasonable adjustment

If further clarification of any medical restrictions and associated temporary reasonable adjustment is required, the RRTWC shall communicate with the treating health practitioner/s, where the workers signed authority has been provided, or via the worker directly.

- This can include requesting completion of physical and/or psychological work capability checklist/s or requesting a medical report from treating medical practitioner/s, to clarify work capacity and restrictions.

- A request for further medical information on work capacity, and temporary reasonable adjustment/s, may be made at any stage during workplace rehabilitation.

To fulfil the duty of care Queensland Health accountability areas hold for workers, it is necessary to seek information relevant to providing a safe place of work. Responding to reasonable adjustment requests may be delayed where required information is not available to make an informed decision about the adjustment requested. Reasonable adjustments may include.

- Temporary modifications to the workplace.
- Time limited supernumerary arrangements to support stay at work/return to work.
- On-the-job training to support participation in temporary, alternate work duties.
- Temporary changes to work practices and roles.

Temporary reasonable adjustment may also include funding an ORP to undertake an assessment and make recommendations for workplace rehabilitation, in accordance with the *Reasonable adjustment HR Policy G3*.

4.3 Stage 3 – Implementing and managing the rehabilitation plan

4.3.1 Implement Stay at Work/RTW arrangements

The line manager shall monitor the workers progress and ensure they are coping with, and complying with, the approved suitable duties.

- Concerns about the workers progress shall be immediately reported by the line manager and/or worker to the RRTWC, who shall notify the treating doctor, with a view to reviewing the program.

4.3.2 Review progress

As suitable duties programs are intended to be temporary and time limited, a cycle of planning, implementing and reviewing shall regularly occur.

Timeframes for review are determined on a case-by-case basis and can be impacted by such factors as progress, nature of injury, medical information provided and operational requirements.

- Wherever possible, timeframes for review shall align with the workers injury type and expected rate of recovery, facilitating a proactive review of their anticipated rehabilitation progression.

4.3.3 Medical clearance certificate

Upon conclusion of participation in workplace rehabilitation a medical clearance certificate should be obtained from the treating doctor, to confirm work capacity for full return to the pre-injury (or other agreed) role.

A medical clearance certificate is used to document that a worker can safely return to their pre-injury (or other agreed) role and is also helpful to ensure correct payment of wages as the worker transitions to their pre-injury (or other agreed) working hours.

4.3.4 Rehabilitation case closure

Rehabilitation case closure shall be considered in such instances as:

- a) The rehabilitation goal has been achieved, and a medical clearance certificate has been received e.g. full return to pre-injury or other agreed role.
- b) A mutually agreed return to work goal has been achieved e.g. hours/duties of pre-injury role permanently modified through agreement by all parties.
- c) Where the suitable duties program is not progressing e.g. hours/duties not increasing towards pre-injury or another agreed role.
- d) Where a safe and sustainable return to work cannot be achieved e.g. significant concerns exist regarding the workers safety or their injury/ill health prevents an outcome for return to work.
- e) Where work incapacity arising from injury/illness persists, but no further workplace rehabilitation involvement is medically indicated.
- f) Where the worker is not actively participating in workplace rehabilitation.

In situations b) – f) outlined above, the rehabilitation case shall be closed and consultation with the relevant Human Resources (HR) unit, rehabilitation contact and the responsible line manager should occur, at which point relevant Department of Health HR policies, such as *Reasonable adjustment HR Policy G3*, and/or *Independent medical examinations of employees HR Policy E11* should be considered.

4.4 Stage 4 – Evaluating and reviewing rehabilitation outcomes

The workplace rehabilitation system shall be subject to an ongoing evaluation and review process.

4.4.1 Workplace rehabilitation outcome and feedback

The RRTWC shall advise all key stakeholders of workplace rehabilitation case closure and document the outcome in the rehabilitation case file.

The RRTWC shall seek feedback at case closure from relevant stakeholders, including the worker and line manager, via any of the following:

- case closure interview/s
- workplace rehabilitation evaluation survey
- other feedback method.

The RRTWC shall summarise and document the feedback derived from the case closure interview, or other feedback methods, and include a copy of the feedback in the workers rehabilitation case file.

The RRTWC shall obtain other relevant case closure documentation, including medical clearance certificate, and record this in the workers rehabilitation case file.

4.4.2 Review outcomes

The RRTWC shall review rehabilitation outcomes and any stakeholder feedback received, to identify any improvements, and ensure the workplace rehabilitation system is subject to ongoing evaluation and review.

4.4.3 Records management and retention

The rehabilitation case file and all related records are to be retained for the stipulated government retention and disposal timeframe (refer also to section 4.4.6).

4.4.4 Where a case transfers to Human Resources (HR) processes

Where work incapacity arising from injury/illness persists and a safe and sustainable return to work cannot be achieved through workplace rehabilitation, consultation with the line manager, rehabilitation contact and HR team should occur so that application of relevant HR policies may instead be considered by the appropriate delegate.

Transfer to the relevant HR unit will include appropriate case file summary and provision of information relevant only to support the ongoing employment of the worker. Noting, except for the purposes of rehabilitation, workers' compensation documents must not be used or obtained for decisions relating to the employment in accordance with s572A of the act.

4.4.5 Managing injury/illness without active workers compensation claim

Queensland Health is committed to the wellbeing of all staff and shall provide workplace rehabilitation to workers with injuries/illnesses, that are not covered by an active workers' compensation claim, where reasonably practicable.

Participation in the workplace rehabilitation process is voluntary for workers who have an injury/illness that is not covered by an active workers compensation claim, except for some compulsory processes.

Assistance shall be provided to workers where compulsory participation has been identified through:

- A process managed under an income protection benefit scheme.
- A process managed by HR under the Public Sector Act 2022 arising from concerns that an employee's absence or unsatisfactory performance is caused by mental or physical illness or disability.

When a line manager becomes aware that a worker is unfit to attend work due to injury/illness, the line manager shall contact the worker to ascertain their medical capacity and advise of the workplace rehabilitation process.

Where a worker with an injury/illness agrees to participate in a structured return to work process, the line manager shall notify the RRTWC and the standard workplace rehabilitation system shall be followed, as outlined in section 4.

Rehabilitation assistance can be provided to workers with non-work-related injuries/illnesses where the following criteria is met:

- The worker has indicated their willingness to participate in a structured rehabilitation and return to work process.
- The worker provides a current medical certificate endorsing a capacity for work and a capacity to safely participate in a rehabilitation and return to work process.
- The worker provides a signed worker authorisation form permitting the RRTWC to directly communicate with their treating doctor and other treating practitioner/s for the purposes of coordinating rehabilitation and return to work. Alternatively, the worker may advise the RRTWC to indirectly communicate with treating practitioners, via the worker obtaining the required medical information, in writing, from their treating practitioner/s.
- The worker agrees to, and complies with, the elements of a suitable duties program as developed by the RRTWC in conjunction with relevant stakeholders.

Where workers with injury/illness, not covered by an active workers compensation claim decline to participate in a structured rehabilitation and return to work process, established leave provisions as outlined in relevant HR policies will apply.

Those workers who decline to participate must provide current medical certificates whilst accessing sick leave entitlements per *Sick Leave HR Policy C64* and maintain communication, at agreed intervals, with their line manager/ other nominated workplace contact during their period of absence from work.

4.4.6 Confidentiality, use and retention of workplace rehabilitation and workers compensation information

RRTWCs are required, in accordance with legislation and the code of conduct for the Queensland public service, to ensure that information on the rehabilitation file is appropriately obtained, used and stored.

A separate rehabilitation file shall be created and maintained for each worker undertaking workplace rehabilitation and should contain copies of all relevant documentation, including medical and allied health reports, correspondence, RTW plans, suitable duties programs, case notes and related documents.

The rehabilitation file for each individual worker shall be stored in a secure environment and managed in accordance with the *Information Privacy Act 2009*, Queensland Health's *Information security policy*, the act and relevant scheme directions.

- Electronic records of the workplace rehabilitation file can be retained in an electronic document and records management system, in accordance with timeframes specified in the Queensland Government's General Retention and Disposal Schedule (GRDS) for administrative records.

Information obtained during rehabilitation must be treated with sensitivity and confidentiality by all parties and in accordance with the *Information Privacy Act 2009* and the code of conduct for the Queensland public service.

If it is necessary to obtain or release information associated with an injured/ill workers rehabilitation, the workers signed authority to obtain or release the information must be obtained.

A copy of the workers signed authority shall be provided to a third party (such as the workers treating doctor and rehabilitation and allied health providers) prior to discussing or exchanging confidential information regarding the employee's injury and rehabilitation program.

An injured/ill worker may request a copy of their workplace rehabilitation file and/or authorise the release of information through written consent to a third party at their discretion.

Should an injured/ill worker request a copy of their workplace rehabilitation file, it must be done so in writing, by either one of the below:

- A direct request to the relevant WHS/HR personnel managing their case, under section 13 of the *Public Sector Regulation 2023*.

- A formal right to information and information privacy access application, under section 24 of the *Right to Information Act 2009*, either via the “approved form” or in writing and accompanied by evidence of the identity of the applicant.

The workers authority is not required for the release of information to the workers’ compensation regulator or the insurer (WorkCover Queensland).

Workers’ compensation documents must not be used or obtained for decisions relating to the employment of the worker, such as deciding whether the employment of a person is to continue or for any process relating to selecting a person for employment, in accordance with section 572A of the act.

- This legislative restriction does not apply to a workers’ compensation document relating to the workers capacity to work, where the document is necessary to secure the workers rehabilitation support, early return to work, or deciding appropriate suitable duties and temporary reasonable adjustments.

Retention and disposal of all administrative records pertaining to workplace rehabilitation shall be in accordance with the Queensland GRDS for administrative records, which includes the following retention timeframes:

- Workplace rehabilitation case file records to be retained for 55 years after business action completed.
- Records submitted or received, relating to workers’ compensation and all other compensation and/or insurance claims, to be retained for seven years after settlement of a claim.
- Records relating to information privacy access applications to be retained for seven years after business action completed.
- Employee complaints/records/decisions to be retained for seven years after business action completed.
- Employee service history (including medical reports) to be retained for 80 years from the employee’s date of birth or seven years from date of separation, whichever is later.
- Legal advice (not involving minors) – (including legal service request, briefs to the legal provider, records of ongoing discussions, revisions/drafts of instructions, legal advice) – 10 years after business action completed.

5 Insurance Protections

All Queensland Health accountability areas hold relevant workers’ compensation insurance cover for workers’ as defined by section 11 (1) of the act, with WorkCover Queensland.

The chief executive of the Hospital and Health Service and the Deputy Director General for the department shall ensure compliance with all relevant legislation as per respective service level agreements and appointed responsibilities.

Those persons not considered to be workers under the act may be covered by other insurance arrangements:

- HHS board members and agency-registered volunteers are covered for personal accident and injury insurance under the existing insurance arrangements held through the treasury managed insurance scheme, the Queensland Government Insurance Fund

(QGIF). Under the QGIF policy, compensation is equivalent to the benefits that would be paid were the insured entitled to compensation under the act, excluding damages entitlements. QGIF does not provide rehabilitation case management services under this arrangement.

- Foundation-registered volunteers require local insurance arrangements to be undertaken, such as a contract of insurance with WorkCover Queensland or other commercial insurer.
- Contractors who are not deemed to be 'workers' as defined under the act, require their own personal injury and illness insurance coverage.
- Work experience program and university placement students are not covered by any personal injury and illness insurance applicable to the department or HHS employees. It is the responsibility of their school, university or training provider to provide any insurance necessary.
- Students undertaking work integrated learning (i.e. a work-based learning opportunity which is assessable by the university or otherwise a requirement of an academic course or program) and work experience placement (i.e. not a course requirement but provision of experience to students as part of their education under the *Education (Work Experience) Act 1996* (Qld)), both meet the definition of 'student' within the meaning of section 22 of the act and consequently are not considered to be unpaid interns under the act.

As part of the annual workers' compensation premium renewal process, and/or within 12 months of a rehabilitation and return to work coordinator's appointment to the role, each Queensland Health policy holder shall ensure the details of all their appointed rehabilitation and RRTWC are provided to WorkCover Queensland, inclusive of RRTWCs' names and contact details, the workplaces each RRTWC has responsibility for and a statement about how the RRTWC is considered to be appropriately qualified to perform the functions of a RRTWC.

Queensland Health workers who reside outside of Queensland for more than six months are not considered Queensland workers under the act. The state of connection (within Australia) or the worker's principal place of employment (if overseas) must be determined, and appropriate workers' compensation insurance arrangements must be put in place to cover these workers.

Queensland Health employees may also be individually insured through either the government superannuation fund, QSuper, or a superannuation fund of their choosing, with personalised insurance entitlements available for death cover and/or total and permanent disability cover, as well as income protection cover for injury and illness, unless they opt out, have a superannuation balance under \$6,000 or are under 25 years of age.

- The nominated income protection insurer will pay a prescribed percentage of the workers insured salary, for injured/ill workers who meet eligibility criteria for income protection benefit entitlements.

6 Workers' compensation entitlements

All Queensland Health workers (permanent, temporary and casual), are eligible to apply for workers' compensation benefits in the event of a work caused injury or illness, whilst:

- at the workers place of employment
- away from the place of employment in the course of the workers employment duties
- temporarily absent from the place of employment during an ordinary recess
- journeying to/from the place of employment.

Workers' compensation eligibility requirements are defined in the act.

It is the role of WorkCover Queensland, as the workers' compensation insurer of Queensland Health accountability areas, to determine liability for workers' compensation claims.

The process for applying for workers' compensation, requesting a review of a workers' compensation claims determination, or appealing the outcome of a review decision, is outlined in the act and is available from WorkCover Queensland.

7 Workplace rehabilitation employee entitlements

Entitlements of workers on workers' compensation or graduated return to work for income protection insurance benefits and health managed cases is a payroll transactional services function.

The workers line manager must complete all necessary forms by payroll cut-off dates to ensure accurate payment of employee entitlements. Further detail can be found via the Payroll Transactional Services, injury and rehabilitation claims intranet pages at <https://qheps.health.qld.gov.au/payroll-assistance/payroll-services/piarct>.

Legislation

- Anti-Discrimination Act 1991
- General Retention and Disposal Schedule (Queensland Government agency administrative records – an authorisation under s26 of the Public Records Act 2002) QDRS: Queensland State Archives: 27 March 2020
- Scheme Directions: Guidelines for standard for rehabilitation second edition: September 2016: Workers' Compensation Regulatory Services, Office of Industrial Relations
- Human Rights Act 2019
- Information Privacy Act 2009
- Managing the risk of psychosocial hazards at work Code of Practice 2022
- Public Records Act 2002
- Public Sector Act 2022
- Public Sector Regulation 2023
- Work Health and Safety Act 2011
- Workers' Compensation and Rehabilitation Act 2003
- Workers' Compensation and Rehabilitation Regulation 2025

Supporting documents

8 Policy

- Work health, safety and wellbeing policy QH-POL-401: Queensland Health

9 Guideline

- Workplace rehabilitation guideline QH-GDL-401-5-2: Queensland Health

10 Forms and templates

- Worker authorisation form
- Workplace rehabilitation – your return-to-work brochure
- Workplace rehabilitation physical capability form
- Workplace rehabilitation psychological capability assessment form
- Workplace rehabilitation suitable duties program
- Workplace rehabilitation progress review form
- Workplace rehabilitation case review and barrier analysis tool
- Workplace rehabilitation evaluation survey

11 Supporting directives and references

- Code of Conduct for the Queensland Public Service: Public Service Commission: January 2011
- Work experience programs and placements HR Policy B3 (QH-POL-262): Queensland Health
- Employees requiring placement HR Policy B36 (QH-POL-237): Queensland Health
- Annual/Recreation leave HR Policy C51 (QH-POL-100): Queensland Health
- Sick leave HR Policy C64 (QH-POL-230): Queensland Health
- Workplace conduct and ethics HR policy E1 (QH-POL-113): Queensland Health
- Preventing and responding to workplace sexual harassment and other unlawful sexual conduct HR Policy E5 (QH-POL-228): Queensland Health
- Independent medical examinations of employees HR Policy E11 (QH-POL-170): Queensland Health
- Individual employee grievances HR Policy E12 (QH-POL-140): Queensland Health
- Workplace harassment HR Policy E13 (QH-POL-266): Queensland Health
- Reasonable adjustment HR Policy G3 (QH-POL-210): Queensland Health
- Information security policy (QH-POL-468): eHealth Queensland
- Information access, use and disclosure (QH-IMP-484-2). Queensland Health Digital Standard
- Corporate Records Management Policy (QH-POL-467): Department of Health
- Minister for Industrial Relations Directive 01/19: Leave without Salary Credited as Service: 13 September 2019

- Director-General approval: Accrual of accrued days off and professional development leave during workers' compensation leave: 16 March 2021
- Records governance policy v1.01: Queensland Government Chief Information Office - Queensland State Archives: April 2019
- ISO 45003: 2021 – Psychological Health and safety at work: International Organisation for Standardisation: June 2021
- Payroll Transactional Services, injury and rehabilitation claims remuneration, benefits and entitlement details (<https://qheps.health.qld.gov.au/payroll-assistance/payroll-services/piarct>)

12 Guides and Manuals

- National Return to Work Strategy 2020-2030: Safe Work Australia: December 2019
- *Position statement: Realising the health benefits of work*: Royal Australasian College of Physicians - Australasian Faculty of Occupational & Environmental Medicine: 2011
- Department of Health Human Resource (HR) Delegations Manual
- Hospital and Health Services Human Resource (HR) Delegations Manual
- *It Pays to Care – Bringing evidence-informed practice to work injury schemes helps workers and their workplaces*: Royal Australasian College of Physicians – Australasian Faculty of Occupational and Environmental Medicine: April 2022
- Wages definition manual: Information for employers: WorkCover Queensland: 30 April 2019 | BK100 v23
- Work capacity certificate – workers' compensation form 132M version 1: 2016
- Work Safe Queensland provides information on compensation claims for workers.
- QSuper Income Protection Benefit Guide (including claim forms): 1 July 2019
- Queensland Government Right to Information and Information Privacy Access Application: V09 January 2020/Form 1 RTI-IP

Definitions

Term	Definition
Accident insurance	Accident insurance is insurance by which an employer is indemnified against all amounts for which the employer may become legally liable, for injury sustained by a worker employed by the employer for— • compensation; and • damages.
Accountability area	Department of Health and each Hospital and Health Service (HHS) are accountability areas within Queensland Health. Queensland Ambulance Service operate within their own Safety Management System and should reference internal policy, procedures and guidelines.
Appropriately qualified	'Appropriately qualified' is defined in the Interpretation Act 1954 as "...having the qualifications, experience or standing appropriate to perform the function..."

Term	Definition
'Appropriately qualified' Rehabilitation and Return to Work Coordinator (RRTWC)	Criteria to be met by current and prospective RRTWCs, under the Workers' Compensation and Rehabilitation Act 2003, in order to perform the functions of a RRTWC under the act.
Damages	Damages are damages for injury sustained by a worker in circumstances creating, independently of the <i>Workers' Compensation and Rehabilitation Act 2003</i>, a legal liability in the workers employer to pay damages to— • The worker; or • if the injury results in the workers death—a dependent of the deceased worker.
Delegate / authorised delegate	The 'delegate' is as listed in the Department of Health and Hospital and Health Services Health Human Resource Delegations Manuals, as amended from time to time.
Department of Health	The Queensland Government's Health Department, encompassing the Office of the Director-General and all corporate divisions. Queensland Ambulance Service is not included for purposes of the intended scope of this document. Hospital and Health Services are included for purposes of the intended scope of this document.

Term	Definition
Employee record	In accordance with section 9 of the <i>Public Sector Regulation 2023</i>: - Each of the following documents is, to the extent the document contains information about a prescribed employee, an employee record— - a report, correspondence item or other document about the employee's work performance, work conduct or work history; - a medical report about the employee; - a written allegation of misconduct by the employee. - However, none of the following documents is an employee record about the prescribed employee— - a medical report about the employee indicating that disclosure of information in the report to the employee might be prejudicial to the employee's mental or physical health or wellbeing. - a document created by an appropriately qualified person for the primary purpose of providing a professional counselling service to another public service employee that contains information about the employee. - a document about the employee that— - is a public interest disclosure, or a record of a public interest disclosure, made under the Public Interest Disclosure Act 2010; or - was brought into existence for the administration of that Act. - A document about the employee relating to suspected corrupt conduct under the Crime and Corruption Act 2001 or an investigation of the conduct. - A document about the employee relating to a suspected criminal offence or an investigation of the offence. - A document that is subject to legal professional privilege.
Employer / persons responsible for workers covered under each workers' compensation insurance policy	The employer/persons responsible for workers covered under each workers' compensation insurance policy refers to an entity of Queensland Health, such as the Department of Health or a Hospital and Health Service. When used in relation to financing rehabilitation costs, it means the local work unit of the injured/ill worker.
Event (in the context of work caused injury/illness)	An event is anything that results in injury or illness, including latent onset injury, to a worker. An event includes continuous or repeated exposure to substantially the same conditions that results in injury/s to a worker, whether the injury/s happen immediately or over a period. (The above definition is utilised by WorkCover Queensland when determining liability / eligibility for workers' compensation entitlements).
Executive leader	The most senior person of each accountability area and can include persons reporting to that position.
Hospital and Health Service (HHS)	A statutory body established under the Hospital and Health Boards Act 2011 responsible for the provision of public sector health services for a geographical area, which includes one or more health facilities.

Term	Definition
Host employment placement	A temporary rehabilitation program conducted in an alternate suitable work environment, when the injured/ill workers presenting medical capacity and/or availability of suitable duties in the pre-injury work environment precludes an early return to work in the pre-injury work environment. A host employment placement may be arranged, for a time limited period, with a different work unit within the Department of Health or a HHS within Queensland Health, or with a different (external) employer, with the aim of returning the injured/ill worker to their pre-injury work unit/position when the host employment rehabilitation program is complete.
Insurer	WorkCover Queensland, Queensland's workers' compensation insurer.
Income protection insurer	QSuper, the Queensland Government superannuation fund and income protection insurer, or other commercial insurer.
Line manager	The manager or supervisor (or delegate) responsible for supervision of the worker in the workplace, and responsible for certain administrative functions, including provision to the payroll service centre of accurate information pertaining to leave taken and hours worked during the injured/ill workers rehabilitation program.
Person-centred approach	An approach that places the injured worker front and centre of their recovery and return to work. Being person-centred occurs when you work in collaboration with an injured worker rather than telling or doing things to them.
Psychosocial hazard	Definition as per section 55A of the WHS Regulation, that is: A psychosocial hazard is a hazard that— • arises from, or relates to— – the design or management of work; or – a work environment; or – plant at a workplace; or – workplace interactions or behaviours; and may cause psychological harm, whether or not the hazard may also cause physical harm.
Psychosocial risk	Definition as per section 55B of the WHS Regulation, that is: A psychosocial risk is a risk to the health or safety of a worker or other person from a psychosocial hazard. (A person conducting a business or undertaking must manage psychosocial risk in accordance with WHS risk management principles, including the hierarchy of control measures, to eliminate or minimise psychosocial risks so far as is reasonably practicable).

Term	Definition
Queensland Health	Queensland Health comprises the Department of Health and the 16 independent Hospital and Health Services (HHSs). Queensland Health refers to the public healthcare sector, incorporating the Department of Health and HHSs. (The Queensland healthcare system incorporates the public, private and not-for-profit healthcare sectors).
Reasonable adjustment	Reasonable adjustment refers to the process where an employer takes account of a person's impairment and makes appropriate temporary or permanent modifications and adjustments to the workplace (including to the employee's role) to lessen the impact of the impairment or enable the employee to commence employment or return to work.
Rehabilitation and return to Work coordinator	A rehabilitation and return to work coordinator is a person who— • is appropriately qualified to perform the functions of a rehabilitation and return to work coordinator under the <i>Workers' Compensation and Rehabilitation Act 2003</i>; and • has the functions prescribed under a regulation. A person is taken to be appropriately qualified to perform the functions of a rehabilitation and return to work coordinator under the <i>Workers' Compensation and Rehabilitation Act 2003</i> if the person has completed a training course approved by the Regulator.
Rehabilitation and return to work Plan	The rehabilitation and return to work plan is, in the case of an active workers compensation claim, developed by the insurer in consultation with all stakeholders. This plan will encompass all elements of the workers rehabilitation program including, but not limited to, long and short term rehabilitation goals, treatment, return to work requirements and suitable duties as applicable, resources and supports required with outcome expectations.
Return to Work (RTW) Hierarchy	Rehabilitation and return to work goals shall be prioritised according to the return to work (RTW) hierarchy wherever feasible, whereby the RTW goal is identified with a preference to the following hierarchy: 1. Return the worker to their usual job, in their usual work area 2. Return the worker to a different job, in their usual work area 3. Return the worker to their usual job, in a different work area 4. Return the worker to a different job, in a different work area 5. Seek a return-to-work option through temporary host employment, alternate duties for the purpose of building work capacities toward pre-injury duties 6. Seek a return-to-work option with a different employer, through the relevant workers' compensation or compulsory third-party insurer, in cases where compensable entitlements exist.
Risk	The possibility that harm (death, injury or illness) might occur when exposed to a physical or psychosocial hazard.

Term	Definition
Suitable duties / suitable duties program	In relation to a worker, are work duties for which the worker is suited, having regard to the following matters: - the nature of the workers incapacity and pre-injury employment - relevant medical information - the rehabilitation and return to work plan for the worker - the provisions of the employer's workplace rehabilitation policy and procedures - the workers age, education, skills and work experience - if duties are available at a location (the other location) other than the location in which the worker was injured/made ill, whether it is reasonable to expect the workers to attend the other location, and - any other relevant matters.
Trauma-informed approach	Trauma-informed approaches seek to avoid re-traumatising the injured/ill worker and empower individuals in decision making, creating safety and trust, equity and respect, choice and collaboration, and rebuilding a sense of control, hope and empowerment.
Unjustifiable hardship	The <i>Anti-Discrimination Act 1991</i> indicates the issue of whether the supply of special services or facilities to a worker would impose "unjustifiable hardship" on an employer depends on all the relevant circumstances of the case, including the: - nature of the special services or facilities - cost of supplying the special services or facilities and the number of people who would benefit or be disadvantaged - financial circumstances of the employer; - disruption that supplying the special services or facilities might cause - nature of any benefit or detriment to all people concerned. Whether there is unjustifiable hardship depends on the circumstances of the particular case. What might be unjustifiable hardship for one employer or workplace might not necessarily be unjustifiable hardship for another.
Worker (for workers' compensation matters)	A person who works under a contract, and in relation to the work, is an employee for the purpose of assessment for PAYG withholding under the Taxation Administration Act 1953 (Cwlth), schedule 1, part 2-5; and is a person who has sustained a work-related personal injury or illness. (The above definition is utilised by WorkCover Queensland when determining liability / eligibility for workers' compensation entitlements). In the context of the workplace rehabilitation system and associated documents, 'worker' can also refer to any worker with an injury or illness who requires rehabilitation support to return to work, which may include those workers with a work-related or non-work-related injury/illness, who are not entitled to and/or are not applying for compensation under the Workers' Compensation and Rehabilitation Act 2003 or an income protection insurance benefits scheme.

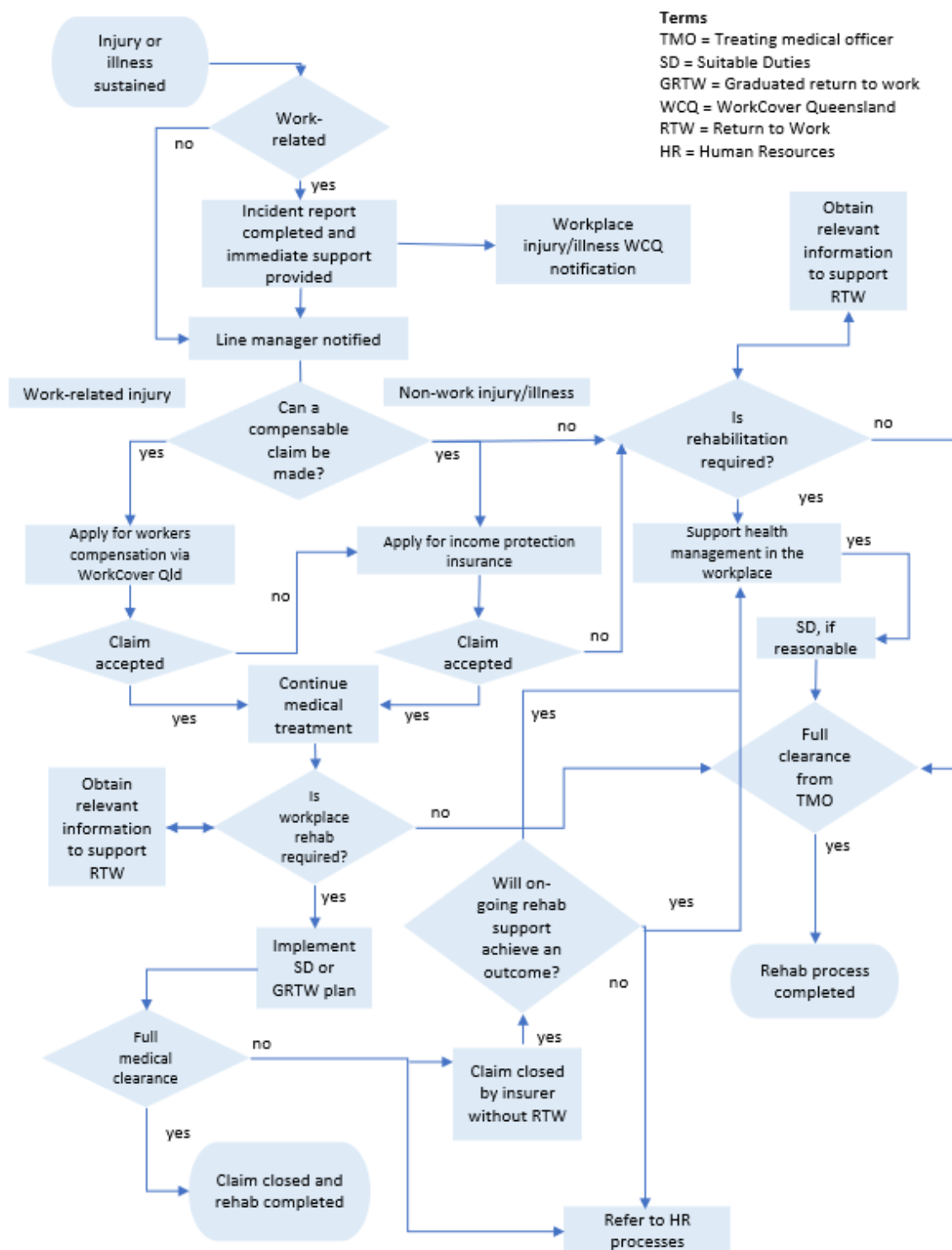
Term	Definition
Workers' compensation	Compensation for work-related injury or illness, that is, amounts for a workers injury payable by an insurer to a worker, a dependent of a deceased worker or anyone else.
Workers' Compensation Regulator	The office of the Workers Compensation Regulator, appointed under the Public Sector Act 2022, must act independently when making a decision under the Workers Compensation and Rehabilitation Act 2003.
Workplace rehabilitation	Rehabilitation of a worker, designed to – ensure the workers earliest possible return to work or stay at work following injury or illness; or maximise the workers independent functioning. Rehabilitation includes– • necessary and reasonable– – suitable duties programs; or – services provided by a registered person; or – services approved by an insurer; or • the provision of necessary and reasonable aids or equipment to the worker. In the context of this standard, rehabilitation refers to all workplace support provided to an ill or injured worker, which may include, but is not limited to, stay at work, return to work and while recovering away from the workplace.
Workplace rehabilitation system	A system of rehabilitation that is initiated or managed by an employer.

History

Date	Change
7 January 2026	Standard review prompted by legislative and other amendments: • amendment of Workers' Compensation and Rehabilitation Act 2003, as outlined in Workers Compensation and Rehabilitation and Other Legislation Amendment Act 2024 • amendment of <i>Workers' Compensation and Rehabilitation Regulation 2025</i>, as outlined in <i>Workers Compensation and Rehabilitation and Other Legislation Amendment Act 2024</i> • introduction of <i>Public Sector Act 2022</i> • introduction of <i>Public Sector Regulation 2023</i> • amendment of <i>Information Privacy Act 2009</i> as outlined in <i>Information Privacy and Other Legislation Amendment Act 2023</i> • amendment of <i>Right to Information Act 2009</i> as outlined in <i>Information Privacy and Other Legislation Amendment Act 2023</i> • introduction of <i>Managing the risk of psychosocial hazards at work Code of Practice 2022</i> • alignment to AS / NZS ISO 45001 criteria • alignment to AS / NZS ISO 45003 criteria • standard reformatted as part of the HR Policy review • amended to update references and naming conventions relevant workplace rehabilitation system • content removed from standard to new Workplace Rehabilitation guideline • relevant remuneration content removed from standard to directly reference QHEPS enable agile information provision • addition of a new attachment to provide workflow detail for suitable duties.
1 July 2021	Workplace Rehabilitation Standard Updating of payroll provisions during workplace rehabilitation Alignment with 2021 Director-General approval: Accrual of accrued days off and professional development leave during workers' compensation leave Updating of insurance coverage arrangements for university placement and work experience program students performing unpaid work in Queensland Health, following the Workers' Compensation Regulator's overruling of WorkCover Queensland's previous (2020) interpretation of this legislative amendment. Updating medical clearance certificate information required as part of workplace rehabilitation.

Date	Change
21 December 2020	Workplace Rehabilitation Standard 2019 legislative amendments Alignment with 2020 Queensland Health Governance Framework
1 January 2017	Workplace Rehabilitation Standard 2015 and 2016 legislative amendments Alignment with Department of Health Policy Framework
30 April 2014	Workplace rehabilitation and return to work Implementation Standard SMS Review project 2013-14

Attachment A: Workplace Rehabilitation Flowchart



Attachment B: Suitable Duties Flowchart

